

SRI LAKSHMI MULTISPECIALTY HOSPITAL

Department of General Medicine • Comprehensive Health Evaluation

SIMULATED / TEST DATA — NOT A REAL MEDICAL RECORD

Patient Information	Synthetic Record
Patient Name	ARUN KUMAR (FICTIONAL)
Medical Record No.	SLMH-SYN-2026-004817
Date of Birth	14 March 1988
Age / Sex	38 years / Male
Date of Registration	12 August 2026
Visit Type	Comprehensive General Health Check-up
Consultant	Dr. Meera Srinivasan, MD (General Medicine)
Department	General Medicine / Preventive Health
Emergency Contact	Not applicable — synthetic dataset

Purpose of this document: This record is intentionally synthetic and designed for software testing, OCR, RAG, clinical-document retrieval, summarization, entity extraction, and question-answering evaluation. Names, identifiers, findings, and results are fictional.

1. Registration & Presenting Concerns

Chief concerns: intermittent fatigue for approximately 3 months; occasional morning headache; episodic acidity/reflux after late meals; dry cough following recent upper-respiratory symptoms.

Item	Details
History source	Patient; considered reliable
Current medications	Occasional paracetamol; no regular prescription medication reported
Allergies	No known drug allergies reported
Tobacco	Denies current smoking
Alcohol	Occasional social use; synthetic history
Sleep	Approximately 6–6.5 hours/night
Diet	Mixed diet; frequent evening tea/coffee; irregular breakfast
Exercise	Walking 2–3 days/week
Family history	Father with hypertension; mother with type 2 diabetes

Review-of-systems screening: No syncope, chest pain, hemoptysis, persistent vomiting, gastrointestinal bleeding, focal neurological weakness, or unexplained acute weight loss reported.

2. Vital Signs & General Physical Examination

Parameter	Observed value	Reference / note
Temperature	98.4 °F	Afebrile at examination
Pulse	78 /min	Regular
Blood pressure	128/82 mmHg	Seated, right arm
Respiratory rate	16 /min	Unlabored
SpO ₂	98% on room air	Normal at rest
Height	171 cm	Measured
Weight	76.2 kg	Measured
BMI	26.1 kg/m ²	Overweight range

General appearance: Alert, oriented, cooperative, comfortable at rest. Hydration clinically adequate. No pallor, cyanosis, icterus, clubbing, pedal edema, or palpable generalized lymphadenopathy.

Skin: Warm and intact. No generalized rash or suspicious pigmented lesion noted during routine examination.

Head/neck: Normocephalic. Oral mucosa moist. No cervical lymphadenopathy. Thyroid not visibly enlarged.

3. Systemic Examination

Cardiovascular

S1 and S2 audible; regular rhythm; no clinically appreciated murmur, gallop, or friction rub. Peripheral pulses palpable and symmetrical.

Respiratory

Chest expansion symmetrical. Vesicular breath sounds bilaterally. No wheeze, crackles, or pleural rub.

Abdomen

Soft and non-distended. Bowel sounds present. No guarding or rebound tenderness. No clinically palpable hepatosplenomegaly.

Neurological

Alert and oriented ×3. Speech normal. Cranial nerve screening grossly intact. Motor strength 5/5 in major muscle groups. Sensation grossly preserved. Gait normal.

Musculoskeletal

No acute joint swelling or deformity. Full active range of movement in major joints.

ENT screening

No active nasal bleeding. Mild intermittent nasal congestion by history. Oropharynx without exudate.

Clinical impression at examination: Stable outpatient presentation. No acute cardiorespiratory or neurological abnormality identified on routine examination.

4. Complete Blood Count

Test	Result	Unit	Lab reference interval
Hemoglobin	14.6	g/dL	13.0–17.0
Hematocrit	43.8	%	40–50
RBC count	5.02	million/ μ L	4.5–5.9
MCV	87.3	fL	80–100
MCH	29.1	pg	27–33
MCHC	33.3	g/dL	32–36
RDW	13.1	%	11.5–14.5
WBC count	7.4	$\times 10^3/\mu$ L	4.0–11.0
Neutrophils	58	%	40–75
Lymphocytes	33	%	20–45
Monocytes	6	%	2–10
Eosinophils	3	%	1–6
Platelets	268	$\times 10^3/\mu$ L	150–450

Laboratory comment: CBC values are within the supplied laboratory reference intervals. No laboratory evidence of anemia, leukocytosis, or thrombocytopenia in this synthetic dataset.

5. Metabolic Screening & Diabetes Evaluation

Analyte	Result	Unit	Reference / interpretation
Fasting plasma glucose	101	mg/dL	70–99 typical normal range
HbA1c	5.6	%	Below prediabetes threshold
Estimated average glucose	114	mg/dL	Calculated
Fasting insulin	11.2	μIU/mL	Lab-dependent
Urea	27	mg/dL	15–45
Creatinine	0.92	mg/dL	0.7–1.3
eGFR	104	mL/min/1.73m ²	>90 generally preserved
Sodium	139	mmol/L	135–145
Potassium	4.3	mmol/L	3.5–5.1
Calcium	9.4	mg/dL	8.5–10.5

Clinical note: Fasting glucose is minimally above the example laboratory normal range, while HbA1c remains below the usual prediabetes range. Lifestyle optimization and routine future monitoring may be considered by the treating clinician.

6. Lipid Profile

Parameter	Result	Unit	Interpretive note
Total cholesterol	198	mg/dL	Near desirable upper range
LDL cholesterol	126	mg/dL	Above optimal for some risk groups
HDL cholesterol	46	mg/dL	Acceptable
Triglycerides	132	mg/dL	Within common target <150
Non-HDL cholesterol	152	mg/dL	Calculated
Total/HDL ratio	4.3	ratio	Calculated

Risk context: Lipid treatment decisions depend on overall cardiovascular risk, blood pressure, diabetes status, family history, smoking status, and other clinical factors; this synthetic report does not constitute a treatment recommendation.

Suggested documentation for follow-up: dietary pattern, physical activity, weight trend, blood-pressure trend, and repeat lipid testing interval as determined by the clinician.

7. Liver Function & Protein Profile

Test	Result	Unit	Reference interval
AST (SGOT)	24	U/L	10–40
ALT (SGPT)	29	U/L	7–56
ALP	82	U/L	44–147
GGT	31	U/L	9–48
Total bilirubin	0.7	mg/dL	0.1–1.2
Direct bilirubin	0.2	mg/dL	0.0–0.3
Total protein	7.2	g/dL	6.0–8.3
Albumin	4.5	g/dL	3.5–5.0
Globulin	2.7	g/dL	2.0–3.5

Laboratory comment: Transaminases, bilirubin, alkaline phosphatase, albumin, and total protein are within the supplied reference intervals.

8. Thyroid, Vitamins & Selected Biochemistry

Test	Result	Unit	Reference interval
TSH	2.14	μIU/mL	0.4–4.0
Free T4	1.18	ng/dL	0.8–1.8
Vitamin B12	412	pg/mL	200–900
25-OH Vitamin D	24	ng/mL	Lab-dependent
Ferritin	88	ng/mL	30–400
Magnesium	2.0	mg/dL	1.7–2.4
Uric acid	6.1	mg/dL	3.4–7.0

Interpretive note: Thyroid function is unremarkable in this synthetic example. Vitamin D is in a low-to-insufficient range according to many commonly used laboratory classifications; interpretation should follow the reporting laboratory's reference standards.

9. Urinalysis

Parameter	Result
Appearance	Clear, pale yellow
Specific gravity	1.018
pH	6.0
Protein	Negative
Glucose	Negative
Ketones	Negative
Blood	Negative
Nitrite	Negative
Leukocyte esterase	Negative
RBC	0–1 / HPF
WBC	0–2 / HPF
Casts	None seen
Bacteria	None significant

Laboratory comment: No significant proteinuria, hematuria, glycosuria, pyuria, or bacteriuria is identified in the synthetic urinalysis.

10. Infectious Disease & Respiratory Screening

Test	Result	Comment
SARS-CoV-2 antigen	Negative	At time of visit
Influenza A antigen	Negative	At time of visit
Influenza B antigen	Negative	At time of visit
Dengue NS1 antigen	Negative	No laboratory evidence in this test
Malaria antigen	Negative	No laboratory evidence in this test
HIV 1/2 Ag/Ab	Non-reactive	Screening result; synthetic
HBsAg	Non-reactive	Screening result; synthetic
HCV antibody	Non-reactive	Screening result; synthetic

Important: Screening tests are not a substitute for clinical assessment. Any real patient with significant symptoms would require testing and interpretation appropriate to the clinical setting.

11. Chest Radiograph Report

Study: Chest X-ray, PA view

Indication: Intermittent dry cough and routine screening.

Findings: Cardiomedial silhouette within normal size limits. Lungs are adequately expanded. No focal air-space consolidation, pleural effusion, or pneumothorax. Mild peribronchial prominence is not identified. No acute osseous abnormality evident on this single projection.

Impression: No acute cardiopulmonary abnormality identified on the provided synthetic radiographic report.

Radiology department: Sri Lakshmi Multispecialty Hospital — Imaging Services

Reported by: Dr. R. Iyer, MD, Radiology (fictional)

Report date: 12 August 2026

12. Abdominal Ultrasound Report

Study: Ultrasound abdomen, complete

Indication: Episodic dyspepsia / preventive health evaluation.

Liver: Normal size and contour. Homogeneous echotexture. No focal lesion. No intrahepatic biliary dilatation.

Gallbladder: Adequately distended. No gallstones or wall thickening.

Common bile duct: Not dilated.

Pancreas: Visualized portions appear unremarkable.

Spleen: Normal size and echotexture.

Kidneys: Normal size and cortical echogenicity. No hydronephrosis or obvious calculus.

Urinary bladder: Grossly unremarkable in the degree of distension provided.

Free fluid: None identified.

Impression: No significant sonographic abnormality identified in this synthetic examination.

13. Gastrointestinal & Symptom-Focused Assessment

Symptom pattern: Burning epigastric discomfort approximately 1–2 times weekly, predominantly after late or spicy meals; occasional sour taste at night. No dysphagia, melena, hematemesis, persistent vomiting, or progressive weight loss reported.

Clinical item	Assessment
Abdominal tenderness	Absent on examination
Bowel habit	Usually once daily; no persistent diarrhea/constipation
Appetite	Preserved
Unintentional weight loss	Denied
Alarm symptoms	Not reported in this synthetic encounter
Working impression	Intermittent dyspeptic/reflux-type symptoms

Plan documented for testing dataset: meal timing and dietary review; avoid lying down immediately after meals; monitor symptom frequency; clinician-directed symptomatic treatment if required; reassessment if alarm symptoms develop.

14. Headache, Fatigue & Neurological Screening

Headache: Intermittent mild morning headache, approximately 1–2 episodes/week, without focal neurological deficit, seizure, syncope, persistent vomiting, or visual loss. Patient reports variable sleep duration.

Fatigue: Gradual, non-disabling fatigue over approximately three months. No fever, night sweats, significant weight loss, or exertional chest pain reported.

Screening domain	Finding
Mental status	Alert, appropriate responses
Cranial nerves	Grossly intact on bedside screening
Motor	5/5 bilaterally in major muscle groups
Sensation	Grossly intact
Coordination	No obvious dysmetria
Gait	Normal
Meningeal signs	Not present

Clinical note: Symptoms are nonspecific. The synthetic record intentionally includes normal examination and laboratory findings to support testing of clinical reasoning and retrieval without implying a definitive diagnosis.

15. Medication & Preventive Health Record

Category	Recorded information
Regular prescription medicines	None reported
OTC medicines	Occasional paracetamol for headache/fever symptoms
Medication allergy	None reported
Immunization history	Not fully documented in synthetic intake
Blood pressure monitoring	Recommended as routine preventive tracking
Dental care	Routine dental review advised
Eye examination	Routine examination advised
Dietary counseling	Balanced meals; reduce excessive refined carbohydrates and late heavy meals
Physical activity	Gradual increase toward regular moderate activity as tolerated
Sleep hygiene	Target consistent sleep schedule and adequate duration

Preventive screening discussion: The record is designed to contain common health-maintenance concepts such as cardiometabolic risk review, weight assessment, lifestyle counseling, and age-appropriate preventive screening prompts.

16. Follow-Up Visit — Symptom Review

Follow-up date: 26 August 2026

Reason: Review of health-check results and fatigue/reflux symptoms.

Parameter	Follow-up
Temperature	98.2 °F
Pulse	76 /min
BP	124/80 mmHg
Weight	75.6 kg
Fatigue	Slightly improved with more consistent sleep
Headache	Less frequent
Reflux symptoms	Reduced after earlier evening meals
Cough	Resolved
New symptoms	None reported

Assessment: Stable outpatient follow-up. No new concerning symptom reported. Review of laboratory and imaging results completed. Lifestyle measures reinforced.

Follow-up plan: Continue lifestyle measures; maintain symptom diary if symptoms recur; repeat preventive laboratory assessment according to clinician-directed interval.

17. Consolidated Clinical Summary

Patient: Arun Kumar (fictional), 38-year-old male presenting for comprehensive general health assessment with intermittent fatigue, mild episodic headache, and reflux-type symptoms.

Examination: Hemodynamically stable with normal cardiopulmonary, abdominal, and neurological screening examination. BMI 26.1 kg/m² at initial visit.

Laboratory profile: CBC, renal profile, liver profile, thyroid function, and urinalysis are broadly unremarkable in the synthetic dataset. Fasting glucose is 101 mg/dL with HbA1c 5.6%. Lipid profile shows LDL 126 mg/dL. Vitamin D is 24 ng/mL.

Imaging: Chest radiograph and abdominal ultrasound reports describe no acute/significant abnormality.

Clinical impression for dataset purposes: Stable preventive-health encounter with nonspecific fatigue and intermittent reflux-type symptoms. No acute abnormality identified.

DATASET NOTICE

This entire document is fictional, generated for testing medical-document processing systems. It is not a patient chart, does not represent a real hospital encounter, and must not be used for diagnosis, treatment, billing, insurance, credentialing, or clinical decision-making.

End of synthetic record.